

Policy on Re-assessment

«Hospital Name»

Document control

Document No.	HCO/AAC/POL/05	Version	2.0
Issue No.	01	Review due	One year from implementation
Date created	_____	Date of implementation	_____
Prepared by	Quality Coordinator Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

After the first assessment, patients are reassessed often enough to see whether treatment is working — the care plan moves with the patient, notes are real clinical notes, and early warning signs trigger prompt action.

1. Purpose

This policy says how «Hospital Name» reassesses patients at appropriate intervals to determine response to treatment and to plan further treatment or discharge; informs out-patients of the next follow-up where appropriate; monitors and modifies the in-patient care plan during re-assessment; requires direct clinical care staff to document re-assessments properly; and lays down guidelines to identify early warning signs of change or deterioration for prompt intervention.

The chapter intent is that patients undergo initial assessment and periodic reassessments resulting in a care plan.

This policy does not cover initial assessment, care plan creation, clinical emergency protocols, or human resource staffing — those are covered in other hospital policies.

2. Scope

This policy applies to treating doctors, nurses and other caregivers who reassess patients at «Hospital Name» in out-patient, day-care, in-patient wards, ICU/high-dependency and areas where patients await admission or a bed. It binds the Medical Superintendent, department heads, Medical Records, and the Quality Coordinator.

It covers the re-assessment of patients at appropriate intervals following initial assessment — including defined reassessment frequency for different patient categories, nursing and medical re-assessment content, re-assessment before procedures, escalation when a patient's condition changes, and the qualifications required to conduct re-assessment.

Boundaries:

- Initial assessment and creation of the care plan are covered in other hospital policies; this policy owns monitoring and modifying that plan at re-assessment.
- The discharge process is covered in other hospital policies; this policy owns re-assessment that informs the decision to discharge.
- Related clinical emergency and acute-care pathways are covered in other hospital policies; this policy owns early-warning identification and escalation.
- Staffing competence and related HR elements are covered in other hospital policies; this policy requires trained staff to use early-warning parameters, not the full HR programme.

3. Policy standards

«Hospital Name» reassesses patients at appropriate intervals to determine response to treatment and to plan further treatment or discharge. Out-patients are informed of their next follow-up where appropriate. For in-patients, the care plan is monitored and modified during re-assessment where necessary. Staff involved in direct clinical care document re-assessments. The organisation lays down guidelines and implements processes to identify early warning signs of change or deterioration for prompt intervention.

«Hospital Name» does not leave a deteriorating patient without escalation when early-warning triggers fire, and does not accept reassessment notes that say only 'patient well' or 'condition better'.

4. Non-negotiable rules

1. Do not go a calendar day without re-assessment of an in-patient by the treating doctor or a doctor from the treating team; reassess more often in ICU and when the condition changes; reassess day-care patients before discharge.
2. Do not send an out-patient away without documenting the next follow-up where a repeat visit is appropriate.
3. Do not leave an ineffective in-patient care plan unchanged when re-assessment shows it must be modified.
4. Do not document re-assessment with only phrases such as 'patient well' or 'condition better' — include vitals, systemic findings and medication orders at minimum (nurses may document vitals).
5. Do not ignore early-warning triggers — escalate to appropriate medical personnel and intervene promptly.
6. Staff who find re-assessment or early-warning rules not followed report it the same shift to the treating doctor or the Medical Superintendent.

5. What we do

5.1 Re-assess at appropriate intervals

After initial assessment, every patient is reassessed periodically and the re-assessment is documented in the case sheet. Re-assessment is done by all applicable caregivers within their scope of practice, registration and applicable laws.

Every patient is reassessed at least once every day by the treating doctor or a doctor from the treating team — this is the organisation's adopted minimum frequency; NABH requires reassessment at appropriate intervals depending on the patient's condition and treatment, without specifying a minimum. Day-care patients are reassessed before discharge. Patients awaiting admission or a bed are also reassessed as applicable.

The hospital defines and documents re-assessment frequencies for each clinical area — including ICU/high-dependency and ward — based on patient condition, clinical need and speciality. Department heads and the ICU in-charge set these frequencies in area-specific guidance.

5.2 Inform out-patients of next follow-up where appropriate

Out-patients are informed of their next follow-up where appropriate. The information is a specific date or a period (weeks/months) and is documented in the medical record or OP consultation sheet.

Follow-up information may be omitted when the patient came only for an opinion or when the condition does not warrant a repeat visit — note that reason briefly in the OP record.

5.3 Monitor and modify the in-patient care plan during re-assessment

For in-patients, during re-assessment the care plan is monitored for effectiveness in achieving the desired results of treatment, care or service. The care plan is dynamic. The treating doctor or a doctor member of the treating team modifies it where necessary according to the patient's condition.

Changes are documented in the medical record. Progress notes, doctor's orders or medication charts may show the change — the change must be findable, not only spoken.

5.4 Direct clinical care staff document re-assessments

Actions taken under re-assessment are documented by the treating doctor or any member of the team per their domain of responsibility. At a minimum, documentation includes vitals, systemic examination findings and medication orders. Nursing staff may document the patient's vitals.

Phrases alone such as 'patient well' or 'condition better' are not acceptable as a re-assessment note.

5.5 Early warning signs — guidelines and prompt intervention

«Hospital Name» lays down guidelines and implements processes to identify early warning signs of change or deterioration in clinical condition and to initiate prompt intervention.

Staff use defined physiological parameters — which may include vital parameters, airway, circulation, neurological status, and any other concerns felt by staff or the patient/family. Parameters may be tailored to speciality and age group.

There is a mechanism to make this information available to appropriate medical personnel to initiate prompt and appropriate actions. The organisation reviews early-warning system effectiveness periodically, with the review interval defined and documented by the Quality Coordinator.

6. Stop-work authority

Do not leave a deteriorating patient without escalation when early-warning signs or defined physiological triggers indicate change or deterioration.

If a trigger is met and appropriate medical personnel have not yet been informed and acted, do not defer escalation for non-urgent work; escalate immediately per the early-warning guideline, start prompt intervention within your scope, and document the trigger, time of escalation, responder and actions.

Stop-work applies to failure to escalate a deteriorating patient. It does not block emergency life-saving measures — those continue while escalation happens.

The person responsible tells the treating doctor and, if there is no timely response, the Medical Superintendent the same shift. Refusing to leave a deteriorating patient unescalated is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for re-assessment intervals, documentation standards and early-warning guidelines. Receives escalation when treating-doctor response is not timely.
Treating doctors / treating team	Daily medical re-assessment at minimum; more often as needed; modify care plans; respond to early-warning alerts.
Nurses and other direct clinical caregivers	Reassess within scope; document vitals and nursing findings; trigger early-warning escalation; never rely on 'patient well' alone.
OPD doctors	Document next follow-up where appropriate.
Department heads / ICU in-charge	Set area-specific frequencies and early-warning parameter sets for speciality and age group.
Quality Coordinator	Audits daily re-assessment, follow-up documentation, care-plan changes, note quality and early-warning effectiveness; reviews early-warning system effectiveness at an interval defined and documented by the

	Quality Coordinator.
Medical Records	Flag missing daily notes and inadequate re-assessment phrases during record review.

8. Quality monitoring (RCA → CAPA)

The Quality Coordinator reviews early-warning system effectiveness at an interval defined and documented by the Quality Coordinator. What is monitored:

- IP sample shows at least daily medical re-assessment; ICU more frequent; day-care re-assessed before discharge.
- OP sample shows follow-up documented where appropriate, or a brief reason when not.
- Care-plan modifications documented when re-assessment required change.
- Re-assessment notes include vitals, systemic findings and medication orders — not only 'patient well'.
- Early-warning triggers escalated with time, responder and action documented; failed escalations treated as incidents.

Root-cause analysis is required when a deteriorating patient was not escalated, or when inadequate re-assessment notes recur; the organisation's defined recurrence threshold applies, as documented in the hospital's Root Cause Analysis procedure.

This policy is reviewed annually, and sooner after any failure-to-escalate event.

9. Training and staff acknowledgement

Doctors, nurses and other direct clinical caregivers are informed of this policy at induction and once a year after that. Training covers interval rules, OP follow-up documentation, care-plan modification, acceptable re-assessment content, early-warning parameters, escalation routes and stop-work.

Staff acknowledgement

I have read the Policy on Re-assessment of «Hospital Name». I will follow the processes described.

Name: _____ Designation: _____

Department / floor: _____ Date: _____

Signature: _____

(One row per staff member. The Quality Coordinator holds signed acknowledgements with clinical induction records.)

10. Distribution

Official master copy: office of the Medical Superintendent, «Hospital Name», with the Quality Coordinator.

Copies issued to: every ward and ICU; day-care; OPD; emergency; nursing administration; medical records.

The current version is available to all staff at the clinical policy file and, if the hospital keeps an intranet, at staff intranet / policies.

When a new version is issued, take old copies out of use.

11. Abbreviations

Abbreviation	Meaning
AAC	Access, Assessment and Continuity of Care (NABH Hospitals chapter)
CAPA	corrective and preventive action
COP	Care of Patients (NABH chapter)
HCO	Hospital (Full Accreditation programme under NABH Hospitals 6th Edition)
HRM	Human Resource Management (NABH chapter)
ICU	intensive care unit
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OE	objective element
OP / OPD	out-patient / out-patient department

12. Traceability to NABH HCO Full Accreditation 6th Edition AAC.5

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

OE	Level	Requirement	Where addressed	Responsible
AAC.5.a	CORE	Patients are re-assessed at appropriate intervals to determine their response to treatment and to plan further treatment or discharge.	Section 3; 5.1	Treating doctors (daily medical re-assessment); nurses and caregivers (within scope); ICU in-charge (ICU frequency)
AAC.5.b	Commitment	Out-patients are informed of	Section 3; 5.2	OPD treating doctors (inform

		their next follow-up, where appropriate.		and document)
AAC.5.c	Commitment	For in-patients during re-assessment, the care plan is monitored and modified, where found necessary.	Section 3; 5.3	Treating doctor or doctor member of treating team (monitor and modify)
AAC.5.d	Commitment	Staff involved in direct clinical care document re-assessments.	Section 3; 5.4	Treating doctors and team members (document per domain); nurses (vitals)
AAC.5.e	Commitment*	The organisation lays down guidelines and implements processes to identify early warning signs of change or deterioration in clinical conditions for initiating prompt intervention.	Section 3; 5.5; Section 6 Stop-work	Medical Superintendent (approve guideline); treating doctors (respond); nurses (trigger); Quality Coordinator (monitor effectiveness)

13. Required Records / Evidence Checklist

Records the hospital holds under this policy, listed by objective element.

AAC.5.a — Re-assessment at appropriate intervals

- IP case sheets showing at least daily re-assessment by treating doctor or treating-team doctor.
- ICU observation charts showing more frequent re-assessment than ward.
- Day-care records showing re-assessment before discharge.

AAC.5.b — Out-patients informed of next follow-up

- OP consultation sheets with next follow-up date or interval documented.
- Sample where follow-up was not applicable with brief reason noted.

AAC.5.c — In-patient care plan monitored and modified

- Progress notes, doctor's orders or medication charts showing care-plan changes.
- Sample where ineffective plan was modified with clinical reason.

AAC.5.d — Re-assessments documented by direct clinical care staff

- Re-assessment notes including vitals, systemic examination findings and medication orders.
- Nursing vital-sign charts contributing to re-assessment documentation.
- Record-review log rejecting notes that only say 'patient well' / 'condition better'.

AAC.5.e — Early warning signs — guidelines and prompt intervention

- Written early-warning guideline with physiological parameters (speciality/age tailored where used).
- Escalation records showing trigger, time informed, responder and actions.
- Early-warning effectiveness review notes (at the interval defined and documented by the Quality Coordinator).
- Incident/RCA records for failure-to-escalate events.

14. References

- National Accreditation Board for Hospitals and Healthcare Providers (NABH), Guidebook to Accreditation Standards for Hospitals, 6th Edition — Access, Assessment and Continuity of Care (AAC), standard AAC.5.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

The requirements in this document are accreditation requirements of the NABH Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. This policy does not import the Consumer Protection Act, 2019, the Clinical Establishments Act, 2010, or the Mental Healthcare Act, 2017 as a checklist. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

«Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed. The clinical and technical content reflects recognised national and international guidance current at the date of preparation.

This document is not issued by, endorsed by, or affiliated with NABH, the World Health Organization, the National Centre for Disease Control, the Food Safety and Standards Authority of India, any Pollution Control Board, or any other body named in it. Wording is original; no text has been reproduced from the standards, rules or guidelines referenced.